Mental Status Examination (MSE)

Psychiatry, Clinical Skills | Dr. Hussain M. Khrad (Consultant Psychiatrist, KFSHRC-Jeddah) | Source: `Mental-Status-Examination.pdf` (41 slides)

> **Legend:** ⭐ = high-yield · *[added]* = not on slides. Images marked "Slide N" come from the lecture; "Added diagram" = drawn for these notes. The 3 MCQ answers and the case model answer come from the lecture's own answer slides.
>
> 🔗 **Bridge from History Taking:**
>
> - The MSE comes **after the 9-section history**, then examination → investigations → formulation.
> - The history covered *what has happened*; the MSE is a **snapshot of now**.
> - The **organic red flags** (visual/olfactory/tactile hallucinations, fluctuating consciousness, disorientation) reappear here as MSE findings.
> - **Suicide/homicide screening** is mandatory again.
> - **Personality Disorders:** schizotypal **ideas of reference** vs delusions of reference; OCPD vs OCD (ego-syntonic vs ego-dystonic) mirrors **obsession vs delusion** here.

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1. Title / Objectives

Domain	Objective	Likely tested as
Knowledge	What the MSE **is and is not**	Snapshot; not an IQ or detailed memory test
Knowledge	⭐ Components **in order**	ABS-M-T-P-C-IJ
Knowledge	⭐ **Mood vs affect · form vs content · illusion vs hallucination vs pseudohallucination**	Definitions and pairs
Skill	Describe each component in correct terminology	Write-up of a manic patient
Skill	⭐ Map **thought-process disturbances** to conditions	Clang/flight → mania; loosening/neologism → schizophrenia
Skill	⭐ Classify **delusions** and **hallucination modalities**	De Clérambault, Othello, Cotard; olfactory → TLE
Skill	Assess **judgement and insight**; document a complete MSE	Insight spectrum
Values	Describe what you **OBSERVE, not infer**; precise, non-pejorative language; MSE as a repeatable measurement	—

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2. Main Content (Lecture Order)

2.1 What the MSE Is and Isn't (Slide 3)

What it is	What it is NOT
The sum of the examiner's **observations and impressions**	**Not an intelligence test**
An estimate of overall **cognitive and emotional** functioning	**Not a detailed memory test**
A **BASELINE** for future comparison and tracking progress	Not a fully precise measure of cognition, affect and behaviour

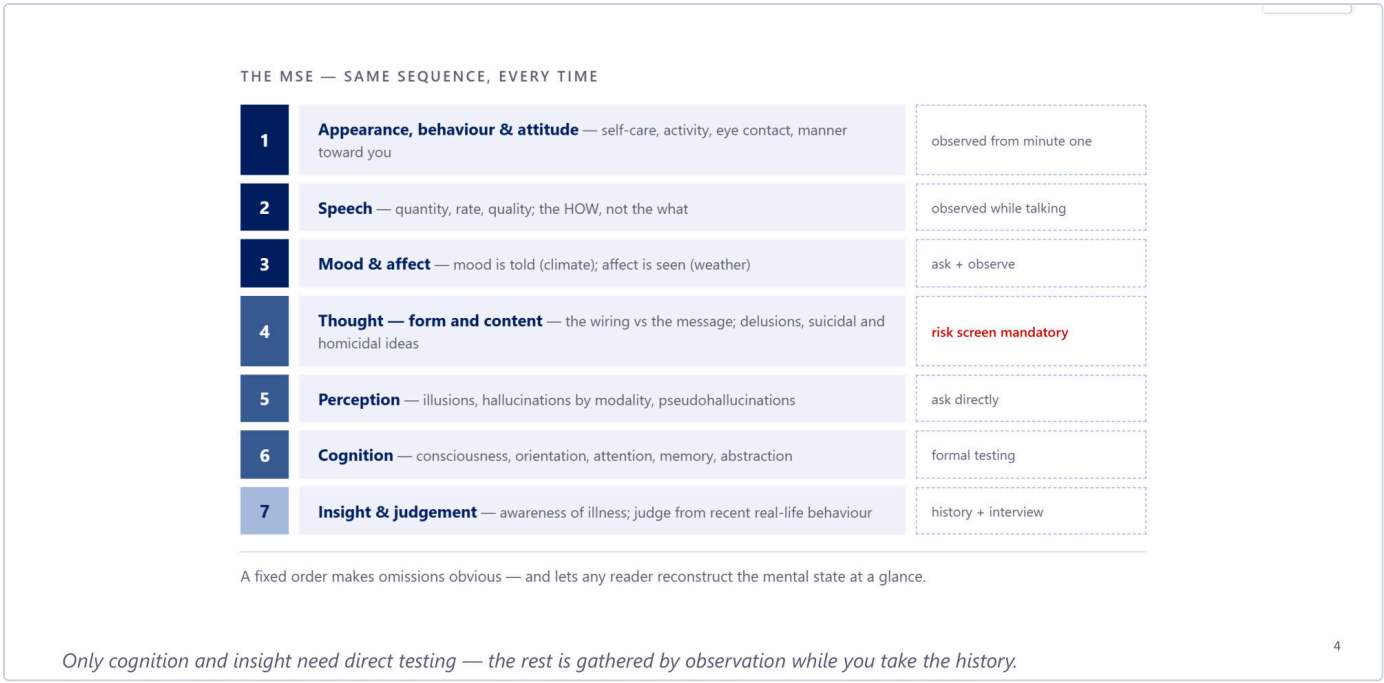
⭐ **A SNAPSHOT IN TIME:** a cross-section of the mental state during *this* interview, which is why it is **repeated** to follow change.

2.2 Components and Method (Slide 4) ⭐

#	Component	Content	How obtained
1	**Appearance, behaviour & attitude**	Self-care, activity, eye contact, manner toward you	Observed **from minute one**
2	**Speech**	Quantity, rate, quality: **the HOW, not the what**	Observed while talking
3	**Mood & affect**	Mood is **told** (climate); affect is **seen** (weather)	Ask + observe
4	**Thought: form and content**	The wiring vs the message; delusions, **suicidal and homicidal ideas**	⭐ **Risk screen mandatory**
5	**Perception**	Illusions, hallucinations by modality, pseudohallucinations	Ask directly
6	**Cognition**	Consciousness, orientation, attention, memory, abstraction	**Formal testing**
7	**Insight & judgement**	Awareness of illness; judge from **recent real-life behaviour**	History + interview

⭐ **Only cognition and insight need direct testing**; the rest is gathered by observation while taking the history. A **fixed order** makes omissions obvious.

Mnemonic *[added]*: "All Smart Medical Students Think Properly, Carefully, Insightfully": **A**ppearance · **S**peech · **M**ood/affect · **T**hought · **P**erception · **C**ognition · **I**nsight/judgement.



Slide 4: MSE components in fixed order

2.3 Appearance, Behaviour & Attitude (Slide 6)

Appearance	Behaviour	Attitude
Body build, **self-care**, clothing, grooming, hair, nails; facial expression; unusual features (e.g. **weight loss**)	Level of activity, posture, **eye contact**; abnormal movements: **tics, grimacing, tremor, disinhibition, hallucinatory gestures**	Manner toward the examiner (verbal, non-verbal): **cooperative, guarded, sarcastic, hostile, seductive, evasive**

Much of the MSE is complete **before you ask a single formal question**.

2.4 Speech: the How, Not the What (Slide 7)

Describe by	Impairments
QUANTITY: talkative, spontaneous, minimal, **poverty of speech**	**Stuttering**; **dysarthria** (an **articulation** problem)
RATE: fast/**pressured** vs slow	⭐ **Pressured speech → mania**
QUALITY: volume, flow, tone, coherence, spontaneity, continuity	**Paucity of speech / increased latency → depression, psychosis**

⭐ Speech = the **FORM** of talking (rate, volume, flow). The **CONTENT** belongs under **Thought**.

2.5 Mood vs Affect (Slides 8–9) ⭐⭐

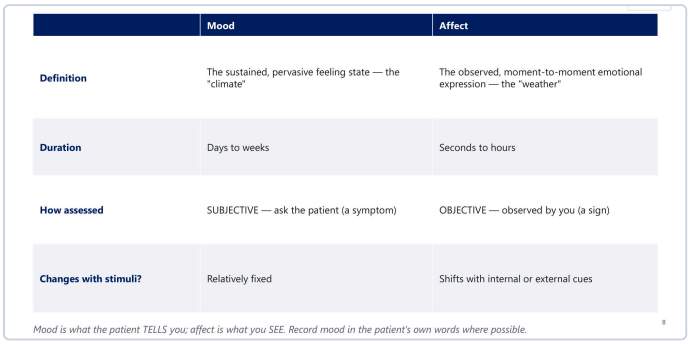
	Mood	Affect
Definition	The **sustained, pervasive** feeling state: the **"climate"**	The observed, **moment-to-moment** expression: the **"weather"**
Duration	**Days to weeks**	**Seconds to hours**
How assessed	⭐ **SUBJECTIVE**: ask the patient (a **symptom**)	⭐ **OBJECTIVE**: observed by you (a **sign**)
Changes with stimuli?	Relatively fixed	Shifts with internal/external cues

Mood is what the patient TELLS you; affect is what you SEE. Record mood **in the patient's own words**.

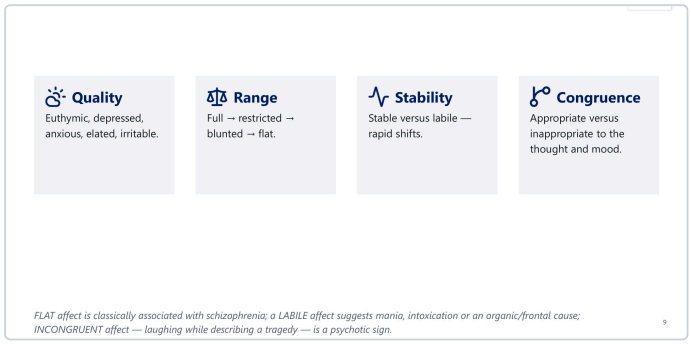
The four descriptors of affect (slide 9):

Descriptor	Options
Quality	Euthymic, depressed, anxious, elated, irritable
Range	**Full → restricted → blunted → flat**
Stability	Stable vs **labile** (rapid shifts)
Congruence	Appropriate vs **inappropriate** to thought and mood

- ⭐ **FLAT** → schizophrenia.
- **LABILE** → **mania, intoxication, organic/frontal** cause.
- **INCONGRUENT** (laughing while describing a tragedy) → **psychotic sign**.



Slide 8: Mood vs affect



Slide 9: The four affect descriptors

> 🔁 **Recap: Observation.** Appearance, behaviour and attitude from minute one. Speech = form (quantity, rate, quality); pressured = mania. Mood = subjective climate (days–weeks); affect = observed weather. Affect has quality, range (full → flat), stability (labile) and congruence.
>
> ▶️ **Up next: Thought and perception, the core of psychopathology.**

2.6 Thought: Form vs Content (Slide 11)

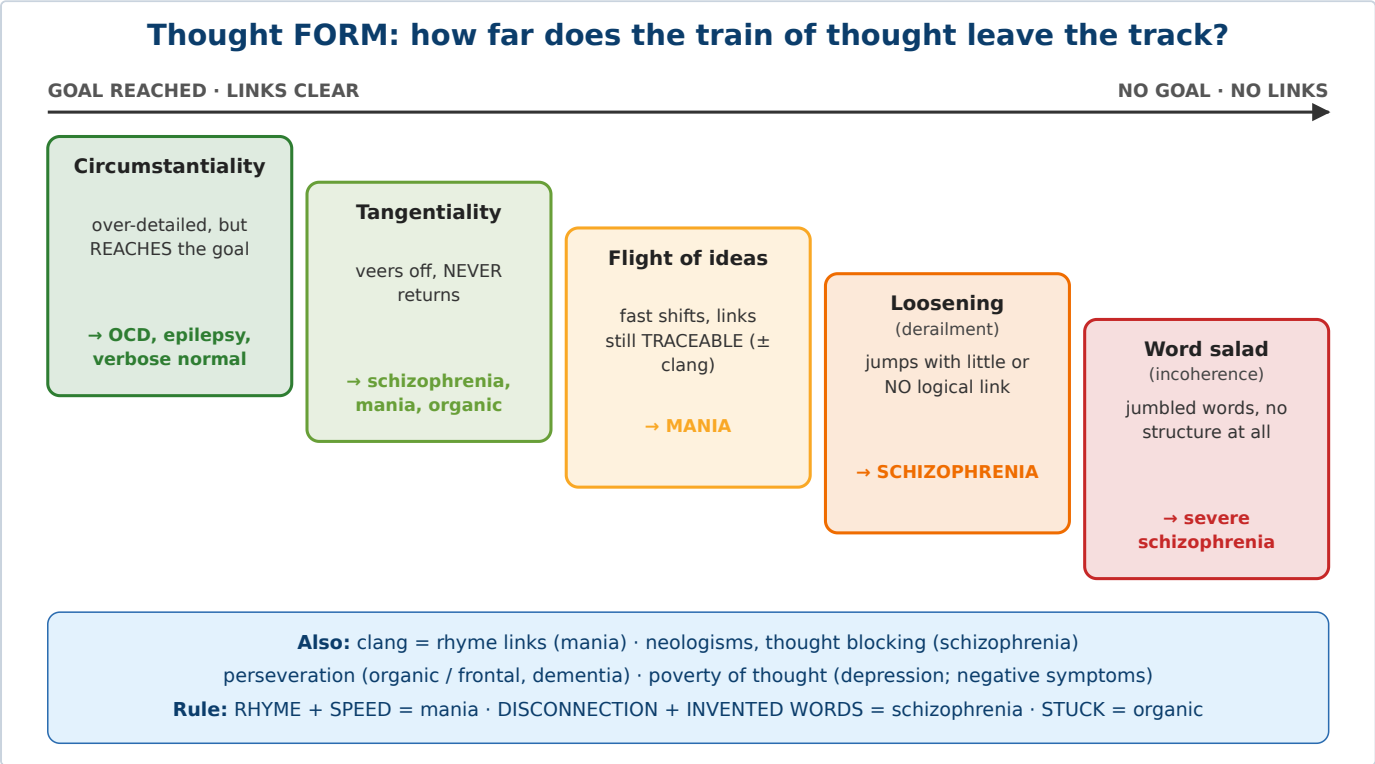
Form / process: HOW they think (the wiring)	Content: WHAT they think (the message)
Flight of ideas	**Delusions**
Loosening of associations	**Obsessions**
Perseveration	**Overvalued ideas**
Circumstantiality, tangentiality, thought blocking	**Suicidal and homicidal ideas**

2.7 Thought-Process Disturbances (Slides 12–15) ⭐⭐

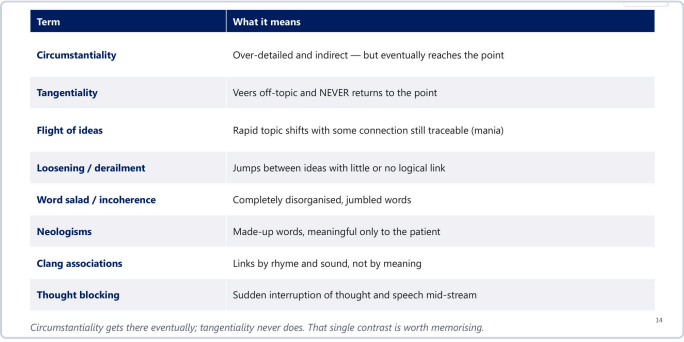
Term	Meaning	Points toward
Circumstantiality	Over-detailed and indirect, **but eventually reaches the point**	**OCD, epilepsy**, or normal in the verbose
Tangentiality	Veers off-topic and **NEVER returns**	Schizophrenia, mania, organic states
Flight of ideas	Rapid topic shifts, **connection still traceable**	⭐ **Mania**
Loosening / derailment	Jumps between ideas with **little or no logical link**	⭐ **Schizophrenia** (formal thought disorder)

Term	Meaning	Points toward
Word salad / incoherence	Completely disorganised, jumbled words	Schizophrenia
Neologisms	Made-up words, meaningful only to the patient	Schizophrenia
Clang associations	Linked by **rhyme and sound**, not meaning	⭐ **Mania**
Thought blocking	Sudden interruption of thought and speech mid-stream	Schizophrenia
Perseveration	*[added definition]* Repeating the same response despite a new question	⭐ **Organic / frontal lobe, dementia**
Poverty of thought/speech	—	**Depression; schizophrenia (negative symptoms)**

- ⭐ **Circumstantiality gets there eventually; tangentiality never does.**
- **Rule:**
  - **RHYME AND SPEED = mania**
  - **DISCONNECTION AND INVENTED WORDS = schizophrenia**
  - **STUCK AND REPETITIVE = organic**



Added diagram: the thought-form spectrum

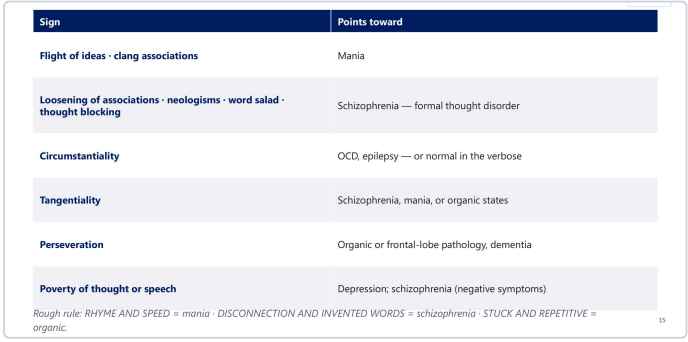


Term	What it means
Circumstantiality	Over-detailed and indirect — but eventually reaches the point
Tangentiality	Veers off-topic and NEVER returns to the point
Flight of ideas	Rapid topic shifts with some connection still traceable (mania)
Loosening / derailment	Jumps between ideas with little or no logical link
Word salad / incoherence	Completely disorganised, jumbled words
Neologisms	Made-up words, meaningful only to the patient
Clang associations	Links by rhyme and sound, not by meaning
Thought blocking	Sudden interruption of thought and speech mid-stream

Circumstantiality gets there eventually; tangentiality never does. That single contrast is worth memorising.

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Slide 14: Thought-process terms



Sign	Points toward
Flight of ideas · clang associations	Mania
Loosening of associations · neologisms · word salad · thought blocking	Schizophrenia — formal thought disorder
Circumstantiality	OCD, epilepsy — or normal in the verbose
Tangentiality	Schizophrenia, mania, or organic states
Perseveration	Organic or frontal-lobe pathology, dementia
Poverty of thought or speech	Depression; schizophrenia (negative symptoms)

Rough rule: RHYME AND SPEED = mania · DISCONNECTION AND INVENTED WORDS = schizophrenia · STUCK AND REPETITIVE = organic.

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Slide 15: Thought disorder → condition

2.8 Thought Content (Slide 16)

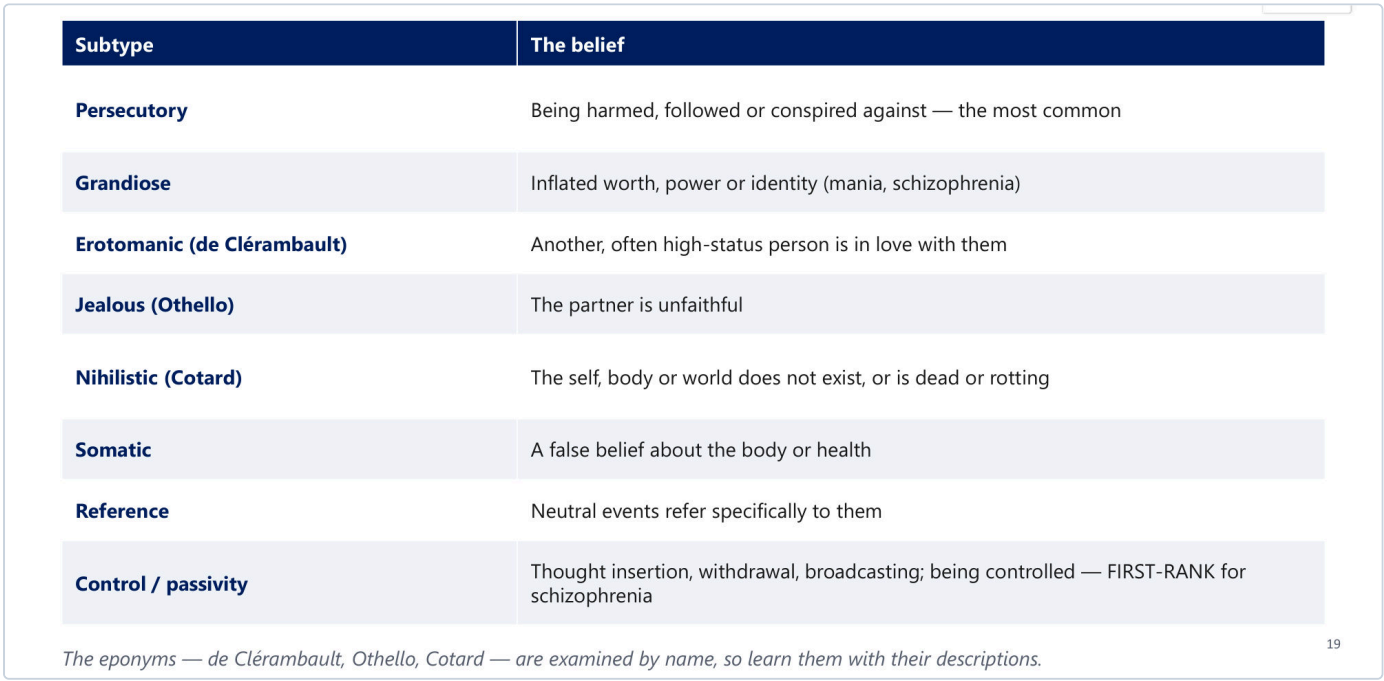
Abnormal beliefs	Preoccupations	⭐ Always ask: risk
DELUSIONS: **fixed, false, unshakeable**	**Obsessions and compulsions**	**SUICIDAL ideation**: intent, plan, means
Overvalued ideas	**Phobias**	**HOMICIDAL** ideation / harm to others
Ideas of reference or influence	**Poverty of thought content**	—

⭐ Suicidal and homicidal ideation screening is a **MANDATORY part of every MSE**.

2.9 Delusion Subtypes (Slides 17–19) ⭐⭐

Subtype	Belief	Eponym / note
⭐ **Persecutory**	Being harmed, followed, conspired against	**The most common**
Grandiose	Inflated worth, power, identity	Mania, schizophrenia
⭐ **Erotomanic**	Another, often **high-status**, person is in love with them	**de Clérambault** syndrome
⭐ **Jealous**	The partner is unfaithful	**Othello** syndrome
⭐ **Nihilistic**	Self, body or world **doesn't exist**, or is dead/rotting	**Cotard** (*[added]* severe psychotic depression, elderly)
Somatic	False belief about the body or health	—
Reference	Neutral events refer specifically to them	—
⭐ **Control / passivity**	**Thought insertion, withdrawal, broadcasting**; being controlled	**FIRST-RANK** for schizophrenia

The eponyms (**de Clérambault, Othello, Cotard**) are examined **by name**.



Subtype	The belief
Persecutory	Being harmed, followed or conspired against — the most common
Grandiose	Inflated worth, power or identity (mania, schizophrenia)
Erotomanic (de Clérambault)	Another, often high-status person is in love with them
Jealous (Othello)	The partner is unfaithful
Nihilistic (Cotard)	The self, body or world does not exist, or is dead or rotting
Somatic	A false belief about the body or health
Reference	Neutral events refer specifically to them
Control / passivity	Thought insertion, withdrawal, broadcasting; being controlled — FIRST-RANK for schizophrenia

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The eponyms — de Clérambault, Othello, Cotard — are examined by name, so learn them with their descriptions.

Slide 19: Delusion subtypes

2.10 Perception: Illusion vs Hallucination vs Pseudohallucination (Slide 20) ⭐⭐

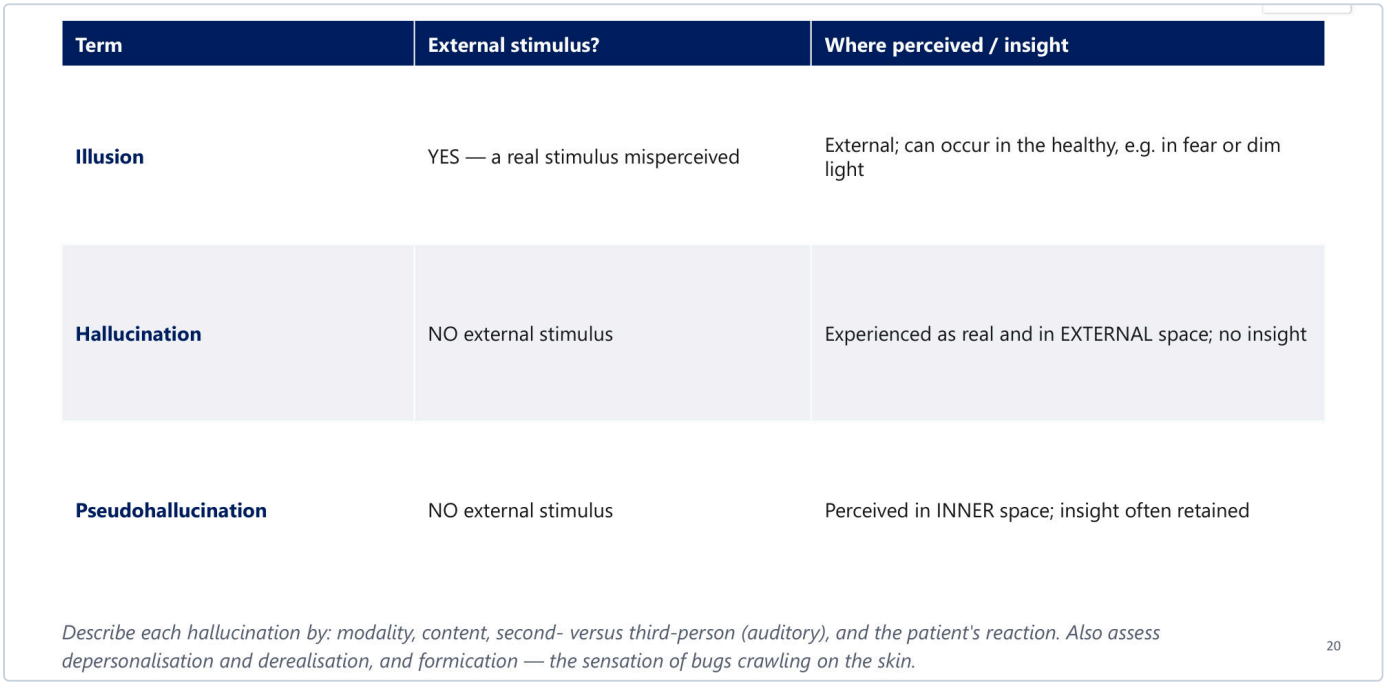
Term	External stimulus?	Where perceived / insight
Illusion	⭐ **YES**: a real stimulus **misperceived**	External; occurs in the **healthy** (fear, dim light)
Hallucination	⭐ **NO** external stimulus	Experienced as real, in **EXTERNAL space**; **no insight**
Pseudohallucination	**NO** external stimulus	Perceived in **INNER space**; **insight often retained**

Describe each hallucination by:

1. **modality**
2. **content**
3. **2nd- vs 3rd-person** (auditory)
4. the **patient's reaction**

Also assess:

- **depersonalisation** and **derealisation**
- ⭐ **formication**: the sensation of **bugs crawling on the skin**



Slide 20: Illusion vs hallucination vs pseudohallucination

2.11 Hallucination Modality → Cause (Slides 21–23) ⭐⭐

Modality	Points toward
⭐ **Auditory**, especially **3rd-person running commentary**	**Schizophrenia** / primary psychiatric illness
⭐ **Visual**	**Delirium, organic disease, substance**
⭐ **Olfactory / gustatory**	**Temporal-lobe epilepsy (aura)**, organic
⭐ **Tactile / formication**	**Cocaine ("cocaine bugs")** or **alcohol withdrawal**

Modality	Points toward
Hypnagogic (falling asleep) / **hypnopompic** (waking)	**Normal**; also **narcolepsy**

⭐ **NON-AUDITORY hallucinations (visual, olfactory, tactile) → organic or substance cause until proven otherwise.**

Lecture case (slides 21–22): a 30-year-old smells **burning rubber**, with intense **déjà vu** and a **rising epigastric sensation**, each episode lasting < 1 minute → ⭐ **temporal-lobe epilepsy** aura.

Modality	Points toward
Auditory — especially 3rd-person, running commentary	Schizophrenia / primary psychiatric illness
Visual	Delirium, organic disease, or substance-related
Olfactory / gustatory	Temporal-lobe epilepsy (aura), organic
Tactile / formication	Cocaine ("cocaine bugs") or alcohol withdrawal
Hypnagogic (falling asleep) / hypnopompic (waking)	Normal phenomena; also narcolepsy

NON-AUDITORY hallucinations — visual, olfactory, tactile — should raise suspicion of an organic or substance cause until proven otherwise.

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Slide 23: Modality → likely cause

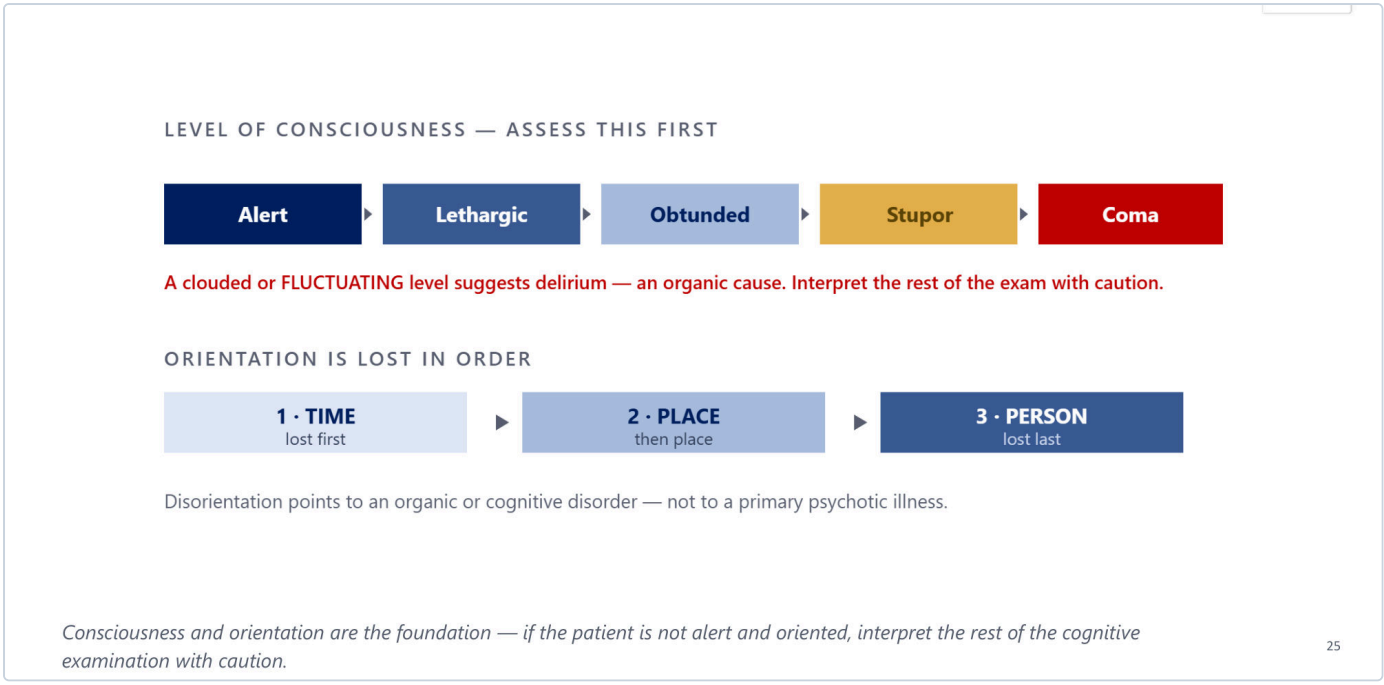
🔁 **Recap: Thought and perception.**

- **Form** = how they think: clang/flight → mania; loosening, neologisms, word salad, blocking → schizophrenia; perseveration → organic.
- Circumstantial reaches the point; tangential never does.
- **Content** = what they think: delusions (persecutory is commonest; de Clérambault, Othello, Cotard; passivity = first-rank), obsessions, overvalued ideas, and **suicide/homicide** screening.
- **Perception:** an illusion has a stimulus; a hallucination has none and is external; a pseudohallucination is internal with insight. Non-auditory → organic.

▶️ **Up next: Cognition, judgement and insight.**

2.12 Consciousness & Orientation (Slide 25) ⭐

- **Level of consciousness (assess FIRST): Alert → Lethargic → Obtunded → Stupor → Coma**.
- ⭐ A **clouded or FLUCTUATING** level suggests **delirium**, an organic cause. Interpret the rest of the exam with caution.
- ⭐ **Orientation is lost in order: 1. TIME** (first) → **2. PLACE** → **3. PERSON** (last).
- **Disorientation points to an organic or cognitive disorder, not a primary psychotic illness.**



Slide 25: Consciousness levels and orientation

2.13 Attention & Memory (Slide 26) ⭐

Domain	Tests
Attention / concentration	⭐ **Serial 7s · spelling "WORLD" backward · digit span**
Registration / immediate memory	**Repeat 3 named objects at once**
Recent memory	**Recall the 3 objects after a few minutes**; recent events
Remote memory	Personal and historical long-term memory

⚠️ **Common pitfall:** "WORLD backward" and serial 7s test **ATTENTION, not registration**. Registration = immediate repetition; recent memory = recall after a delay.

2.14 Language, Aphasia & Visuospatial Function (Slide 27)

Test language **if brain pathology is suspected**.

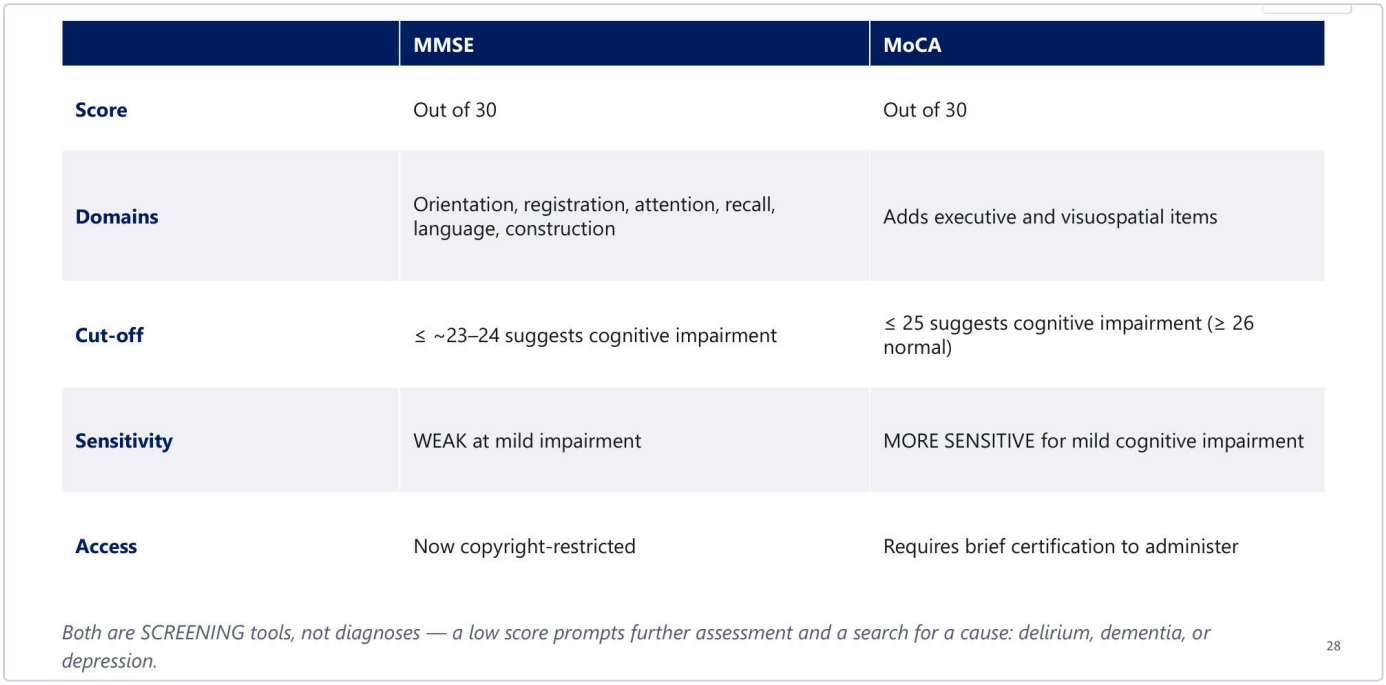
Aphasia	Fluency	Comprehension	Repetition	Lesion
⭐ **Broca's (expressive)**	**Non-fluent**	**INTACT**	Impaired	**Frontal**
⭐ **Wernicke's (receptive)**	**Fluent but nonsensical**	**Poor**	*[added]* Impaired	**Temporal**
Conduction	Fluent	Good	⭐ **Impaired** (the defining feature)	*[added]* Arcuate fasciculus
Nominal (anomic)	Fluent	Good	Good	Naming difficulty (pen, watch)

Visuospatial: copy **interlocking pentagons** or a **clock**. Impairment → **parietal** or organic pathology.

2.15 MMSE vs MoCA (Slides 28–29) ⭐

	MMSE	MoCA
Score	/30	/30
Domains	Orientation, registration, attention, recall, language, construction (a **verbal** core)	⭐ **Adds executive and visuospatial** items (trails, cube, clock)
Cut-off	⭐ **≤ ~23–24** suggests impairment	⭐ **≤ 25** suggests impairment (**≥ 26 normal**)
Sensitivity	**WEAK** at mild impairment	⭐ **MORE SENSITIVE for mild cognitive impairment**
Access	Now **copyright-restricted** (PAR)	Requires brief **certification**
Origin	Folstein, 1975	Nasreddine, 2005

Both are **SCREENING tools, not diagnoses**. A low score prompts a search for **delirium, dementia or depression**.



	MMSE	MoCA
Score	Out of 30	Out of 30
Domains	Orientation, registration, attention, recall, language, construction	Adds executive and visuospatial items
Cut-off	≤ ~23–24 suggests cognitive impairment	≤ 25 suggests cognitive impairment (≥ 26 normal)
Sensitivity	WEAK at mild impairment	MORE SENSITIVE for mild cognitive impairment
Access	Now copyright-restricted	Requires brief certification to administer

Both are SCREENING tools, not diagnoses — a low score prompts further assessment and a search for a cause: delirium, dementia, or depression.

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Slide 28: MMSE vs MoCA

Both score out of 30 — but only the MoCA tests executive and visuospatial function.

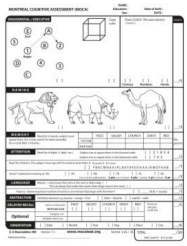
VERBAL



MMSE

Verbal core — orientation, recall, language

+ EXECUTIVE



MoCA

Adds trails, cube and clock (executive & visuospatial)

Shown zoomed-out for recognition, not for administration. MMSE: Rovner & Folstein 1987, courtesy CME Outfitters — the instrument is now copyright-restricted (PAR). MoCA v7.1 © Z. Nasreddine MD, mocatest.org — free for educational use with permission. Reproduced here for internal teaching only.

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Slide 29: MMSE and MoCA forms (for recognition only)

2.16 Abstract Thinking (Slide 30)

- **Abstraction:** the ability to handle concepts and draw appropriate inferences.
- **Tests:**
  - **Similarities/differences** ("how are a car and a train alike?")
  - **Proverbs** ("people in glass houses should not throw stones")
- A **concrete, literal** answer ("the stones will break the glass") → impaired abstraction.
  - Seen in **schizophrenia, dementia, low IQ**.
  - **Normal in children.**
- ⚠️ **Cultural/language caution:** an **unfamiliar proverb produces a concrete answer in a healthy person.**

2.17 Judgement (Slide 31)

- Judgement is the capacity to make sound decisions and act appropriately.
- ⭐ It is **best judged from RECENT REAL-LIFE BEHAVIOUR**. **Hypothetical scenarios** ("what would you do if you found a stamped envelope?") are **less reliable**.
- Impaired **judgement**, not just insight, **drives much of the risk** in mania, psychosis and intoxication.

2.18 Insight: a Spectrum (Slide 32) ⭐

Full	Partial	Impaired
Recognises the symptoms **as illness**	Acknowledges problems but **not as illness**	**Denies** illness or problems
Able to modify behaviour	Knows others see them as ill	No understanding of others' concerns
Cooperates fully with treatment	Variable behaviour change	**Poor treatment adherence**
→ **best adherence and prognosis**	—	→ ⭐ **predicts non-adherence and relapse**

Describe where the patient sits; don't just tick present/absent. "Insight absent" tells the next clinician nothing about how likely the patient is to stop medication.



Slide 32: Insight spectrum

🔁 **Recap: Cognition, judgement and insight.**

- Consciousness first (alert → coma; fluctuating = delirium).
- Orientation is lost in the order time → place → person.
- Serial 7s and WORLD backward test **attention**; 3 objects test registration then recall.
- Aphasias: Broca (non-fluent, comprehension intact), Wernicke (fluent nonsense, poor comprehension), conduction (repetition), nominal.
- **MoCA ≤ 25** is better for MCI; **MMSE ≤ 23–24**.
- A concrete proverb answer → schizophrenia, dementia, low IQ (or unfamiliarity).
- Judgement: from real-life behaviour. Insight: a spectrum; poor insight predicts relapse.

▶️ **Up next: Integration.**

2.19 Concept Distinctions to Know Cold (Slide 34) ⭐⭐

Pair	Key difference
Illusion vs hallucination	Illusion = **real stimulus misperceived**; hallucination = **no stimulus**
Hallucination vs pseudohallucination	**External space, no insight** vs **inner space, insight retained**
Obsession vs delusion	Obsession is **EGO-DYSTONIC, resisted, insight kept**; a delusion is **held as true, no insight**
Overvalued idea vs delusion	Overvalued = strongly held but **understandable**, **not fixed or bizarre**
Mood vs affect	**Sustained, subjective (symptom)** vs **momentary, observed (sign)**

These **5 pairs** account for a large share of MSE exam questions.

Pair	Key difference
Illusion vs hallucination	Illusion = a real stimulus misperceived; hallucination = no stimulus at all
Hallucination vs pseudohallucination	External space and no insight, versus inner space with insight retained
Obsession vs delusion	Obsession is EGO-DYSTONIC, resisted, insight kept; a delusion is held as true with no insight
Overvalued idea vs delusion	Overvalued = strongly held but understandable, and not fixed or bizarre
Mood vs affect	Sustained and subjective (a symptom), versus momentary and observed (a sign)

These five pairs account for a large share of MSE exam questions — know them cold.

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Slide 34: Five distinctions

2.20 Documenting an MSE: Worked Example (Slide 35)

> *"A dishevelled young man, poor eye contact, restless. Speech rapid and pressured. Mood 'on top of the world'; affect elated and labile. Thought form shows flight of ideas; content includes grandiose delusions ('I've invented a cure'); no suicidal or homicidal ideation elicited. No hallucinations. Alert and oriented ×3; attention reduced on serial 7s. Insight impaired; judgement poor."*

→ reads as an **acute manic episode**. Always use the **same fixed order**.

2.21 Integrative Case: Mr. Y, 24 (Slides 36–37) ⭐

Presentation:

- brought to the ED by his father
- **appearance:** bright orange shirt, mismatched shoes, **sunglasses indoors**, unshaven, body odour
- **behaviour:** paces, interrupts, **over-familiar** (uses your first name)
- **speech:** loud, rapid, hard to interrupt
- **thought:** hospital → football → business plan → father's driving; **each link understandable**, but the topic never returns
- **mood/affect:** says "**excellent, the best of my life**"; laughs, briefly tearful, then laughs again
- **content:** "**chosen to reform the national education system**", **already emailed the ministry**
- denies voices; **sleeps 2 h/night for a week**; fully oriented
- says the only problem is his father; doesn't need treatment

Question	Model answer (lecture)
1. Appearance, behaviour, attitude	Brightly and bizarrely dressed, sunglasses indoors, **poor self-care**; **psychomotor agitation**; **over-familiar, disinhibited, intrusive**
2. Speech; mood vs affect	Speech ↑ rate and volume, **pressured**, hard to interrupt. **MOOD** (subjective, sustained) = **elated**. **AFFECT** (objective) = **labile and expansive**
3. Thought form	⭐ **FLIGHT OF IDEAS**: rapid shifts with **comprehensible connections**. In loosening the links **cannot be followed** → schizophrenia

Question	Model answer (lecture)
4. Thought content	⭐ **GRANDIOSE DELUSION**: fixed, false, unshakeable, out of keeping with background, and **ACTED UPON**
5. Judgement / insight	**Judgement impaired; insight ABSENT**: denies illness and **externalises** onto his father

Twist: the single feature that would push the diagnosis toward schizophrenia → **links that cannot be followed** (loosening), rather than merely rapid ones.

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3. High-Yield Comparison Tables

Table A: Mania vs Schizophrenia on the MSE *[synthesised from slides 9, 15, 19, 23, 35–37]*

MSE domain	Mania	Schizophrenia
Appearance	Bright, bizarre, flamboyant	Neglected, odd
Speech	**Pressured**, loud	Poverty, latency, neologisms
Affect	**Elated, labile, expansive**	**Flat/blunted, incongruent**
Thought form	**Flight of ideas, clang**	**Loosening, word salad, blocking, neologisms**
Content	**Grandiose**	**Persecutory**, **passivity (first-rank)**
Perception	Mood-congruent if present	**3rd-person auditory, running commentary**
Orientation	Intact	Intact

Table B: Organic Clues on the MSE

Domain	Organic clue
Consciousness	**Clouded/fluctuating**
Orientation	**Disoriented** (time first)
Perception	**Visual, olfactory, gustatory, tactile**
Thought form	**Perseveration**
Affect	**Labile** (frontal)
Cognition	Visuospatial (parietal) or aphasia

Table C: Classic Hallucination Pairs

Clue	Answer
Burning rubber + déjà vu + epigastric rising	**Temporal-lobe epilepsy**
Bugs crawling under the skin	**Cocaine** or **alcohol withdrawal**
Voices discussing the patient in the 3rd person	**Schizophrenia**
Elderly, fluctuating, seeing children/animals	**Delirium** (or Lewy body dementia *[added]*)
Seeing figures while falling asleep, otherwise well	**Hypnagogic**: normal (or narcolepsy)

Table D: Memory and Attention Tests

Test	Domain
Serial 7s, WORLD backward, digit span	**Attention**
Repeat 3 objects now	**Registration**
Recall 3 objects after minutes	**Recent memory**
Personal history, historical events	**Remote memory**
Pentagons, clock	**Visuospatial** (parietal)
Proverbs, similarities	**Abstraction**

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4. Rules, Exceptions, Patterns

- **Only cognition and insight are formally tested**; everything else is observed.
- **Speech = form; thought content = what is said.**
- **Mood = subjective (in the patient's words); affect = objective.**
- **Flat → schizophrenia · labile → mania/intoxication/frontal · incongruent → psychosis.**
- **Rhyme + speed = mania · disconnection + neologism = schizophrenia · stuck = organic.**
- **Circumstantial reaches the goal; tangential never does.**
- **Persecutory is the most common delusion. Passivity phenomena are first-rank.**
- **Illusions occur in healthy people**; hypnagogic/hypnopompic hallucinations are **normal**.
- **Non-auditory hallucinations → organic/substance.** Auditory 3rd-person → schizophrenia.
- **Disorientation → organic, not primary psychosis.** Lost in the order time → place → person.
- **WORLD backward = attention, not registration.**
- **MoCA (≤ 25) > MMSE (≤ 23–24) for mild impairment.** Both are screens only.
- **Concrete proverbs:** schizophrenia, dementia, low IQ, children, **or an unfamiliar proverb**.
- **Judgement from real behaviour > hypotheticals.**
- **Insight is a spectrum; poor insight → non-adherence and relapse.**
- **Suicide/homicide screening in EVERY MSE.**

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5. Clinical Correlations

- **ED patient with flight of ideas and grandiosity** → mania. Impaired judgement drives risk (spending, driving).
- **Patient with formication after stopping heavy drinking** → alcohol withdrawal (*[added]* risk of delirium tremens). In a cocaine user → "cocaine bugs".
- **Episodic olfactory hallucination with déjà vu** → EEG/neurology, not antipsychotics.
- **Elderly inpatient disoriented to time, with visual hallucinations** → delirium. Consciousness and orientation first.
- **Discharge summary:** describe insight on the spectrum (partial vs impaired) to flag relapse risk.
- **Proverb testing in Saudi practice:** use culturally familiar proverbs, or similarities, to avoid false "concrete" answers.

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6. Rapid Revision (Last-Day)

1. **MSE = a snapshot**; a baseline; repeated. Not an IQ or memory test.

2. **Order:**
    1. appearance/behaviour/attitude
    2. speech
    3. mood & affect
    4. thought (form + content; **risk**)
    5. perception
    6. cognition
    7. insight & judgement
3. Only **cognition and insight** are directly tested.
4. **Speech:** quantity, rate, quality. **Pressured = mania**; latency = depression/psychosis; dysarthria = articulation.
5. **Mood:** subjective, days–weeks (climate). **Affect:** objective, seconds–hours (weather).
6. **Affect:** quality, range (full → restricted → blunted → flat), stability (labile), congruence.
7. **Flight/clang → mania** · **loosening/neologism/word salad/blocking → schizophrenia** · **perseveration → organic** · **circumstantiality → OCD/epilepsy** · **poverty → depression/negative symptoms**.
8. **Delusions:**
    - persecutory (commonest)
    - grandiose
    - erotomanic (**de Clérambault**)
    - jealous (**Othello**)
    - nihilistic (**Cotard**)
    - somatic
    - reference
    - **passivity = first-rank**
9. **Illusion** = real stimulus; **hallucination** = none, external, no insight; **pseudohallucination** = inner, insight.
10. **Modalities:** - auditory 3rd-person → schizophrenia - visual → delirium/organic - olfactory → TLE - tactile → cocaine/alcohol withdrawal - hypnagogic → normal/narcolepsy
11. **Consciousness:** alert → lethargic → obtunded → stupor → coma. **Orientation lost:** time → place → person.
12. **Attention:** serial 7s, WORLD backward, digit span. **Registration:** immediate 3 objects. **Recent:** delayed recall.
13. **Broca** non-fluent/comprehension intact (frontal) · **Wernicke** fluent/poor comprehension (temporal) · **conduction** repetition · **nominal** naming.
14. **MMSE ≤ 23–24 · MoCA ≤ 25** (MoCA adds executive/visuospatial; better for MCI).
15. **Concrete proverbs:** schizophrenia, dementia, low IQ, children.
16. **Judgement:** from recent real-life behaviour. **Insight:** full / partial / impaired.
17. **Five pairs:** illusion/hallucination · hallucination/pseudo · obsession/delusion · overvalued/delusion · mood/affect.

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7. Exam-Style Questions

Q1 (lecture, slides 12–13). An elated, over-talkative patient answers in rapid rhyming phrases: "I feel real, the deal is real, the bell will tell." What is this? A. Neologism · **B. Clang association** · C. Circumstantiality · D. Thought blocking

Answer: B (lecture key).

- Linked by **sound and rhyme**, not meaning.
- With elation and pressured speech → **mania**.

Q2 (lecture, slides 17–18). A 28-year-old woman is unshakeably convinced that a famous singer she has never met is secretly in love with her and sends coded messages through his songs. The delusion is? A. Grandiose · **B. Erotomanic (de Clérambault)** · C. Delusion of reference · D. Nihilistic

Answer: B (lecture key).

- The belief that a higher-status person loves her = **de Clérambault**.
- The "coded messages" are an **added delusion of reference**.

Q3 (lecture, slides 21–22). A 30-year-old has brief episodes of smelling burning rubber, with intense déjà vu and a rising epigastric sensation, each lasting < 1 minute. Most likely cause? A. Schizophrenia · **B. Temporal-lobe epilepsy** · C. Alcohol withdrawal · D. Cocaine intoxication

Answer: B (lecture key). Olfactory hallucination + déjà vu + epigastric aura = classic **TLE** aura. This is organic.

Q4. A patient asked to spell "WORLD" backward answers "D-L-O-R-W" correctly. Which domain has been tested? A. Registration · B. Recent memory · **C. Attention/concentration** · D. Remote memory

Answer: C (slides 26, 38).

- WORLD backward, serial 7s and digit span test **attention**.
- Registration = immediate repetition of 3 objects.

Q5. A man washes his hands 50 times a day, knows the fear of contamination is "silly," and tries hard to resist. A second man insists, without evidence, that his wife is unfaithful and cannot be persuaded otherwise. Which statement is correct? A. Both are delusions · **B. The first is an obsession (ego-dystonic, insight kept); the second is a delusion of jealousy (Othello)** · C. Both are overvalued ideas · D. The first is a delusion; the second is an obsession

Answer: B (slides 19, 34, 38).

- **Obsession:** resisted, insight retained.
- **Delusion:** held as true, no insight.
- Delusional jealousy = **Othello syndrome**.

Lecture board one-liners (slide 38):

#	Question	Answer
1	Fluent but nonsensical speech, poor comprehension?	**Wernicke's (receptive), temporal**
2	Believing a spouse is unfaithful on no evidence?	**Othello syndrome**
3	"Spell WORLD backward" tests?	**Attention/concentration**, NOT registration
4	Tactile "bugs crawling": two causes?	**Cocaine** and **alcohol withdrawal**

#	Question	Answer
5	Obsession vs delusion?	Obsession: ego-dystonic, resisted, insight kept; delusion: held as true, no insight

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8. Exam Pitfalls & Traps

Trap	Correct thinking
Mood = what you observe	❌ **Mood is subjective** (patient's words); **affect is observed**
Recording speech content under "Speech"	❌ Speech = **form**; content goes under **Thought**
Flight of ideas = loosening	❌ Flight: links **traceable** (mania); loosening: links **not followable** (schizophrenia)
Circumstantiality = tangentiality	❌ Circumstantial **reaches** the point; tangential **never** does
WORLD backward = memory	❌ **Attention**
Every hallucination = psychosis	❌ **Illusions** and **hypnagogic/hypnopompic** experiences are normal; non-auditory → **organic**
Visual hallucinations → schizophrenia	❌ **Delirium/organic/substance**
Disorientation in "psychosis"	❌ Points to **organic/cognitive** disorder; primary psychosis keeps orientation
Obsession = delusion	❌ Obsession is **ego-dystonic with insight**; delusion has **no insight**
Overvalued idea = delusion	❌ Overvalued = understandable, **not fixed or bizarre**
MMSE is the best screen for MCI	❌ **MoCA** (≤ 25) is more sensitive; MMSE is weak in mild impairment
A low MMSE/MoCA = dementia	❌ A **screen**; search for delirium, dementia, depression
Concrete proverb = schizophrenia	⚠️ Also dementia, low IQ, children, and **unfamiliar proverbs** (culture/language)
Judging judgement from hypotheticals	❌ **Recent real-life behaviour** is more reliable
"Insight absent" is enough	❌ Describe the **spectrum** (full/partial/impaired); poor insight predicts relapse
Skipping suicide/homicide in a "routine" MSE	❌ **Mandatory** in every MSE
Wernicke's = non-fluent	❌ Wernicke's is **fluent**, with poor comprehension; **Broca's** is non-fluent, comprehension intact